

The Execution Sequence: Why Order Is Not Optional

The six pillars of healthcare transformation — Policy, Economics, Technology, Operations, Clinical, and Equity — are often presented as a framework of simultaneous imperatives. Each pillar is necessary. Each pillar depends on the others. Remove any one and the system fails. This is true. But it obscures a critical practical question that every health system leader, state official, and reform architect must answer before a single dollar is spent or a single care model is redesigned: in what order do you actually build this?

The answer is not arbitrary. The sequence is determined by dependency logic — the hard reality that some things cannot exist until other things are already in place. Getting the order wrong does not merely slow progress; it produces structural failures that are expensive to reverse and politically difficult to explain. A state that invests in clinical redesign before its payment architecture is set creates a care transformation with no financial sustainability. A system that deploys operational infrastructure before its technology is built creates a workforce managing paper in a data-free environment. A reform that attempts to correct equity gaps before a functioning system exists has nothing to calibrate against.

This chapter makes the sequencing logic explicit. It draws on the theoretical structure of the six-pillar framework and on the practical experience of states that have undertaken serious transformation efforts, including Vermont's decade-long journey under the Blueprint for Health and the AHEAD Model. The goal is not to provide a rigid prescription — real-world implementation is always messier than any framework — but to give decision-makers a principled basis for staging their investments and managing the dependencies that determine whether transformation succeeds or stalls.

The Chicken-and-Egg Problem: Technology and Economics

Before walking through the full sequence, it is worth pausing on the tension that most frequently confuses reform architects: the relationship between Technology and Economics. The confusion is understandable, because it runs in both directions.

On one hand, you cannot manage economics without technology. Global budget accountability requires data systems that track utilization, attribute patients to providers, calculate total cost of care, and generate the performance reports on which payment adjustments depend. A Total Cost of Care model that lacks the data infrastructure to measure total cost of care is not a payment model — it is an aspiration.

On the other hand, you cannot fund and deploy technology without economics. Technology infrastructure at the scale required for population health management — clinical registries, care management platforms, SDOH screening systems, attributed patient databases — requires sustained investment. Someone must decide to fund it, and that decision is an economic and political act that precedes the build.

The resolution to this apparent paradox lies in distinguishing between two different activities that the word 'economics' conflates:

Economics as design — the policy and analytical work of defining the payment architecture, setting the global budget parameters, negotiating the value-based contracts, and specifying what the system is trying to achieve financially. This work requires analytical capacity and data from existing systems, but it does not require the full technology infrastructure of the reformed system. It precedes the technology build.

Economics as management — the ongoing operational work of running a global budget, monitoring performance against targets, adjusting payments, and holding accountable. This work absolutely requires technology. It cannot happen without the data infrastructure already in place.

The sequencing logic therefore holds: Economics-as-design comes before the technology build, because the design defines what the technology needs to do. Technology comes before Economics-as-management, because

management requires the infrastructure to already be running. In practice, these phases overlap — early technology builds begin while payment design is still being finalized — but the directional dependency is clear: you design the economics first, then build the technology to manage it.

The Six Stages of Execution

With that foundation established, the full execution sequence follows a logic that is both theoretically coherent and empirically grounded in what reform-leading states have actually done.

Stage 1: Policy — Establishing the Authority to Act

Nothing in healthcare transformation can be authorized, funded, built, or deployed without the legal and regulatory framework that makes it permissible. This is not a bureaucratic observation — it is a structural one. The authority to operate a global budget, to require attribution of patients to primary care providers, to waive federal Medicaid rules, to mandate data reporting from hospitals and practices — all of this flows from statute, regulation, and federal agreement. Without it, reform architects are proposing things that no one has the legal power to do.

Policy therefore comes first not because it is the most important pillar in some abstract ranking, but because it is the prerequisite for all other action. The AHEAD Model's authority to operate derives from a federal agreement with CMS. Vermont's Blueprint for Health derives its mandate from state statute. The Rural Health Transformation Program's funding derives from legislative appropriation. Each of these policy foundations had to be established before the economic, technological, operational, clinical, or equity work they enabled could begin.

The practical implication for reform architects is this: before investing in payment design, technology procurement, or workforce deployment, confirm that the legal authority to do so is secured. Time spent on operational planning without policy foundation is time that must often be redone once the political and legal landscape becomes clear.

Stage 2: Economics — Designing the Payment Architecture

Once legal authority is established, the next imperative is to design the payment architecture that will govern the transformed system. This means answering the questions that define what the entire reform is trying to achieve: What is the unit of accountability — an individual patient, an attributed population, a geographic region? What is being measured and rewarded — cost containment, quality improvement, specific outcome metrics? Over what time horizon are performance obligations defined? How are savings shared and risks allocated?

These are not implementation questions. They are foundational design questions, and they must be answered before technology systems can be specified, before operational infrastructure can be designed, and before clinical redesign can be prioritized. A technology vendor building a care management platform needs to know what the payment model is measuring. A hospital designing its care coordination program needs to know what outcomes it will be held financially accountable for. A clinical team redesigning primary care delivery needs to know whether the economic logic rewards prevention, reduces hospitalization, or manages total cost — because the clinical model follows from the economic incentive.

This is also the stage at which the funding commitment for the technology build must be secured. The payment architecture design must include an explicit answer to the question: what data infrastructure is required to manage this model, and where does the money to build it come from? Failing to answer this question at Stage 2 consistently produces the same failure mode at Stage 3 — a technically sophisticated payment model with no information system capable of running it.

Stage 3: Technology — Building the Operating System

Technology is the operating system on which everything else runs. This is the insight that most traditional healthcare reform sequencing frameworks miss, and it is the insight that distinguishes successful transformation efforts from those that stall at implementation.

Consider what the downstream stages require. Operations requires workforce management systems, credentialing platforms, care coordination software, and administrative infrastructure. Clinical redesign requires patient registries, risk stratification tools, care gap identification, SDOH screening platforms, and the data feeds that tell clinicians which patients need what interventions. Equity requires the population-level data to identify disparities, the screening tools to surface social determinants, and the reporting infrastructure to hold the system accountable for closing gaps. None of these are possible without technology already in place.

The practical challenge is that technology builds are slow, expensive, and politically unglamorous. The temptation is to defer the technology investment and begin the more visible work of operational deployment and clinical transformation. This temptation should be resisted. Systems that deploy care coordination staff before their care management platform is operational create a workforce doing manual work that technology was supposed to automate — and then face the cost and disruption of a mid-stream technology implementation on top of an already-stretched operational infrastructure.

The technology build at Stage 3 does not need to be complete before Stage 4 begins. In practice, operational deployment and technology build proceed in parallel, with early operational capacity running on interim tools while the full infrastructure is developed. But the foundational technology decisions — platform selection, data architecture, interoperability standards, attribution logic — must be made and the build must be underway before operational scale-up begins.

Stage 4: Operations — Deploying the Infrastructure

Operations is where the reform becomes real in the physical world. Workforce is hired, credentialed, and deployed. Care coordination networks are stood up. Administrative infrastructure — billing, contracting, reporting, compliance — is built and tested. The organizational relationships between primary care practices, hospitals, behavioral health providers, and community organizations that the payment model requires are established and formalized.

Operations runs on the technology infrastructure built in Stage 3. The care coordinators use the care management platform. The Hub and Spoke model for substance use treatment depends on the registry that identifies patients in need. The community health teams use the SDOH screening data to prioritize outreach. When the technology is not ready, operations staff improvise — and improvisation at scale produces inconsistency, inefficiency, and burnout.

The operational stage is also where the workforce capacity constraints that are endemic to rural and underserved health systems become most visible. Vermont's Rural Health Transformation Program confronts directly the reality that you cannot redesign care delivery in a geography where there are not enough clinicians to deliver care. Operational planning must therefore include an honest assessment of workforce supply, and where supply is insufficient, workforce development must be treated as an operational prerequisite to clinical redesign — not an afterthought.

Stage 5: Clinical — Redesigning Care Delivery

Clinical redesign is where the economic return on the entire investment is realized. It is the stage at which payment reform, technology infrastructure, and operational capacity converge to change what actually happens between a clinician and a patient. The Patient-Centered Medical Home transforms primary care from acute episodic treatment to proactive population management. The Collaborative Care Model integrates behavioral health into

primary care settings. The Certified Community Behavioral Health Clinic expands access for the highest-need populations. The Blueprint's 5.8:1 return on investment from primary care investment is not a theoretical projection — it is the measured outcome of a clinical model that the preceding stages made possible.

The reason clinical redesign comes fifth rather than first is not that it is less important. It is that clinical transformation without economic alignment reverts. Clinicians redesign their practice in response to the incentives they face and the infrastructure available to support them. Without a payment model that rewards the redesigned care, the transformation is unsustainable. Without the technology to identify which patients need what interventions, the care model is operating blind. Without the operational infrastructure to coordinate across settings, the redesign is isolated to individual practices rather than expressed at the population level.

The sequencing logic here is one of the most empirically robust findings from a decade of payment reform research: clinical transformation that precedes payment reform consistently fails to sustain. Clinical transformation that follows payment reform, enabled by technology and operational infrastructure, produces the outcomes the reform was designed to achieve.

Stage 6: Equity — Calibrating and Correcting the System

Equity is last in execution sequence, but it must be understood clearly: last in sequence is not last in priority. The decision to place equity at Stage 6 is a structural observation about when meaningful equity interventions become possible, not a statement about the relative importance of equity as a value or an objective.

The reason equity comes last is that equity interventions — at the systemic level — require a functioning system to audit, correct, and hold accountable. Health equity is not achieved by adding a screening question to an intake form. It is achieved by identifying which populations the system is failing, understanding the structural reasons for that failure, and redesigning the system's policies, practices, and resource allocations to produce different outcomes. That work requires the data infrastructure to measure disparities at the population level. It requires the operational infrastructure to deploy targeted interventions in the geographies and populations where gaps are largest. It requires the clinical infrastructure to deliver culturally competent, linguistically accessible care at scale.

Vermont's equity challenges illustrate the point. Essex County's 8% uninsurance rate — among the highest in the state — is not primarily an equity problem that can be solved by equity interventions alone. It is a primary care access problem, a workforce problem, and an economic problem. The equity intervention — targeted outreach, expanded coverage, language access — becomes effective only when the access, workforce, and economic foundations are in place to receive the newly engaged population. Equity without infrastructure produces demand that the system cannot meet.

This does not mean equity considerations are deferred until Stage 6. On the contrary: equity must be embedded as a design principle in every preceding stage. The policy framework must include equity mandates. The payment architecture must include equity metrics and accountability structures. The technology build must include the data infrastructure to measure disparities. The operational deployment must prioritize underserved geographies. The clinical redesign must integrate cultural competence. What happens at Stage 6 is the systematic audit of whether those embedded commitments have produced equitable outcomes — and the corrective action when they have not.

The Sequence in Practice: Overlap, Iteration, and Political Reality

The six-stage sequence described above is a logical structure, not a rigid timeline. In practice, transformation efforts begin multiple stages simultaneously, encounter political and fiscal constraints that force reordering, and learn from early implementation in ways that require revisiting earlier-stage decisions. This is normal and expected.

What the sequence provides is not a prohibition on parallel work but a framework for understanding dependencies. When a technology build stalls, the operational deployment that depends on it must slow or accept the cost of

interim workarounds. When a payment architecture is revised mid-implementation, the clinical models built around it must be re-evaluated. When a workforce shortage surfaces at Stage 4, the clinical redesign at Stage 5 must be scoped to what the available workforce can deliver.

The most common sequencing failure in healthcare transformation is not a deliberate choice to invert the stages — it is the accumulation of political pressures that push visible, high-profile clinical initiatives ahead of the foundational infrastructure that makes them sustainable. A new governor wants to announce a landmark primary care transformation. A hospital system wants to demonstrate its commitment to value-based care. A federal program requires rapid implementation to meet grant deadlines. Each of these pressures creates incentives to begin clinical work before the economic, technological, and operational foundations are ready.

The role of the reform architect — the health system leader, the state official, the policy designer — is to hold the sequence with discipline while managing the political reality that demands visible progress at every stage. This requires the ability to distinguish between genuine parallel progress (early technology builds running alongside payment design) and premature sequencing (clinical transformation launched before payment reform is operational). The framework presented in this chapter provides the analytical basis for making that distinction.

Sequencing Summary

Stage	Pillar	Key action	Why this sequence	Vermont anchor
1	Policy	Establish legal authority and regulatory framework	Nothing can be authorized, funded, or built without statutory and regulatory permission	AHEAD Model agreement with CMS; Acts 167 and 68; state waiver authority
2	Economics	Design payment architecture and secure funding commitments	Payment reform defines what the system is trying to achieve and funds the technology infrastructure required to manage it	Total Cost of Care model design; global budget parameters; Blueprint per-member-per-month payment structure
3	Technology	Build data infrastructure and clinical information systems	Technology is the operating system on which operations, clinical redesign, and equity interventions all run — it cannot be added after the fact	Clinical registry development; SDOH screening platforms; care management information systems; AI-assisted documentation tools
4	Operations	Deploy workforce, administration, and care coordination infrastructure	Operational systems — staffing, credentialing, care coordination networks — run on top of the technology infrastructure already in place	Blueprint Hub and Spoke infrastructure; Community Health Team deployment; Rural Health Transformation workforce expansion
5	Clinical	Redesign care delivery models and transform clinical practice	Care redesign is where the economic ROI is actually realized — but only because payment	PCMH practice transformation; Collaborative Care Model expansion; CCBHC development; Blueprint 5.8:1

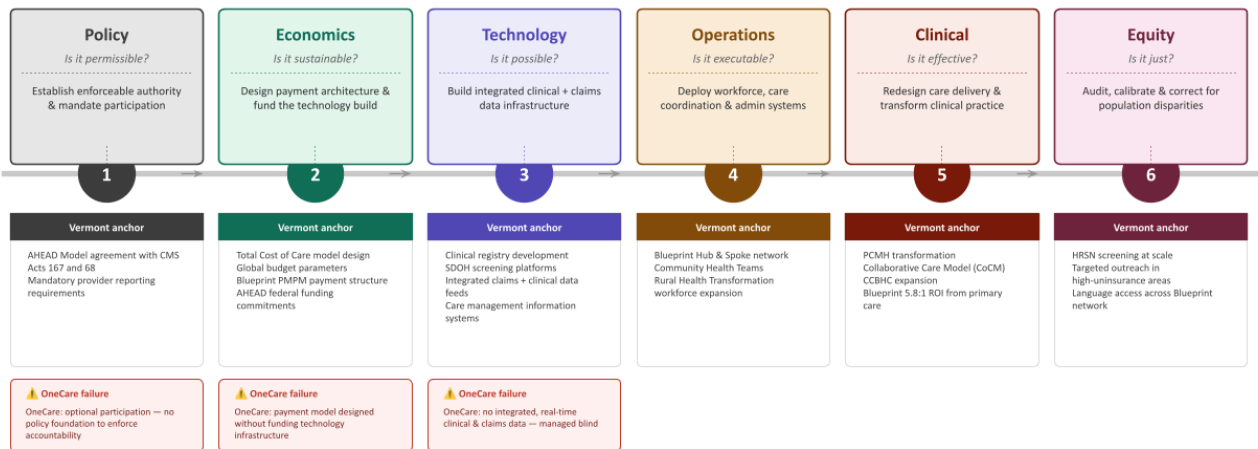
Stage	Pillar	Key action	Why this sequence	Vermont anchor
			incentives, technology infrastructure, and operational capacity are already aligned	ROI from primary care investment
6	Equity	Apply equity interventions to calibrate and correct the operating system	Equity is not last in importance — it is last in execution because meaningful equity interventions require a functioning system to audit, correct, and hold accountable	HRSN screening at scale; targeted outreach in high-uninsurance geographies; language access and cultural competence integration across the Blueprint network

The table above summarizes the execution sequence, the key action at each stage, the dependency rationale, and the Vermont anchor that illustrates each stage in practice. It is intended as a reference tool for reform architects who need to explain sequencing logic to governance bodies, legislative committees, and clinical leadership who may not immediately understand why the foundational stages must precede the visible ones.

The single most important message of this chapter is also the simplest: the order in which healthcare transformation is built is not a project management preference. It is a structural requirement determined by the dependencies between the pillars. Respecting that order does not guarantee success. Ignoring it reliably produces failure.

The Six-Pillar Execution Sequence

Why order is not optional — and what happens when it is ignored



Upper cards: key action at each stage | Lower cards: Vermont implementation anchors | Red flags: OneCare sequencing failures (Stages 1–3)